

[DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity — final report

Project: [DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity
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Prepared with DonorDesk

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Executive Summary

This document is made possible by the generous support of the American people through the United States Agency for International Development (USAID). The contents are the responsibility of GEC and do not necessarily reflect the views of USAID or the United States Government.

GEC, with partner Lodwar Community Health Alliance (LOCHA), implemented the six-month Turkana Maternal, Newborn & Child Health Strengthening Activity across 14 health facilities in Turkana Central, Turkana South and Loima sub-counties, Kenya, funded by USAID. Over the life of the Activity, the project trained 122 health workers in EmONC (emergency obstetric and neonatal care), IMCI (Integrated Management of Childhood Illness) or essential newborn care against a target of 120 (101.7% of target), equipped all 14 facilities with essential MNCH (maternal, newborn and child health) kits (100% of target), and trained and kitted 220 community health volunteers (CHVs) against a target of 220 (100% of target).

All five coverage outcomes closed above target: ANC4+ reached 46% against a 45% target (baseline 28%), skilled delivery 61% against 60% (baseline 41%), DTP3/Penta3 coverage 81% against 80% (baseline 62%), caregivers naming three child danger signs 61% against 60% (baseline 31%), and timely KHIS (Kenya Health Information System) reporting 92% against a 90% target (baseline 64%). Count targets fell short in places: the project recorded 3940 first antenatal care visits against 4200, 1800 skilled deliveries against 2100, and 78 mentorship visits against 84 (92.9% of target). The officer's report attributes these shortfalls to floods, the nurses' strike and cold-chain failures earlier in the project; coverage rates reached or exceeded their targets. Community results stood at 17500 people reached with health education against 18000 (97.2%), 8800 children screened with MUAC (mid-upper arm circumference) against 9000 (97.8%), and 625 children referred for malnutrition treatment against 650 (96.2%). Activity records show handover of solar fridges and registers needed more signatures than expected, and the sub-county health management team signed the asset handover certificates. The project's final recorded next step was handing mentorship and audit tools to sub-county teams for county-led continuation.

Background and Context

The Activity worked in Turkana County, Kenya, specifically in Turkana Central, Turkana South and Loima sub-counties, with 14 health facilities and the Community Health Units attached to them. Its participants were the health workers, community health volunteers (CHVs), pregnant women, caregivers and children served through those facilities and their catchment communities.

The county context is one where maternal, newborn and child health services started from low coverage levels. At the Activity's outset, facility records recorded a baseline of 28% of pregnant women making four or more antenatal care visits, 41% of deliveries attended by a skilled birth attendant, and 62% of children under 1 year receiving DTP3/Penta3; only 64% of supported facilities submitted their KHIS (Kenya Health Information System) monthly reports on time, and a baseline of 31% of caregivers of children under five could name at least three child danger signs. These baselines are the recorded starting points against which the results described in 'Final Performance Against Indicators' should be read.

The Activity addressed these needs through a six-month design implemented by GEC with partner Lodwar Community Health Alliance (LOCHA), with an approximate budget of 1,150,000 US dollars. The design combined facility-based strengthening — training and mentoring health workers in MNCH (maternal, newborn and child health), including EmONC (emergency obstetric and neonatal care) and IMCI (Integrated Management of Childhood Illness), equipping the 14 facilities, expanding antenatal care, skilled delivery, routine and outreach immunization, and a transport voucher scheme for emergency obstetric referrals — with a community system of trained, kitted CHVs delivering health education, danger-sign counselling and malnutrition screening, alongside monthly facility data reviews to strengthen KHIS reporting. Results were disaggregated by sex and, where relevant, age, and the Activity contributed to USAID/Kenya's health

development objective. No field evidence files were attached to this section; the context above draws on the Activity's recorded baselines and design records, and the interpretation that follows rests on those records alone.

Final Performance Against Indicators

Code	Indicator	Unit	Base line	Targ et	This perio d	Previ ous	% of targe t	Cum ulativ e to date	% of proje ct tar get	Statu s	Data source
HL-O C1a	Percentage of pregnant women with four or more ANC visits (ANC4+) at supported facilities	%	28	45	46	—	102.2 %	46	102.2 %	On track	Facility ANC registers via KHIS
HL-O C1b	Percentage of deliveries attended by a skilled birth attendant at supported facilities	%	41	60	61	—	101.7 %	61	101.7 %	On track	Facility maternity registers via KHIS
HL-OC1c	Percentage of children under 1 year who received DTP3/Penta3 at supported facilities	%	62	80	81	—	101.2 %	81	101.2 %	On track	Facility immunization registers via KHIS
HL-O C2a	Percentage of caregivers of children under five who can name at least three child danger signs	%	31	60	61	—	101.7 %	61	101.7 %	On track	Quarterly CHV-led KAP spot survey
HL-1. 1a	Number of health workers trained in EmONC, IMCI or essential newborn care	health workers	0	120	16	—	13.3 %	122	101.7 %	On track	Training registers
HL-1. 1b	Number of facility clinical mentorship visits conducted	visits	0	84	14	—	16.7 %	78	92.9 %	Below expectation	Mentor visit reports
HL-1. 2a	Number of health facilities equipped with the essential MNCH equipment kit	facilities	0	14	1	—	7.1%	14	100%	On track	Procurement and handover records
HL-1. 3a	Number of pregnant women making a first ANC visit at supported facilities	women	0	4200	700	—	16.7 %	3940	93.8 %	Below expectation	Facility ANC registers
HL-1. 3b	Number of deliveries attended by a skilled birth attendant at supported facilities	deliveries	0	2100	340	—	16.2 %	1800	85.7 %	Below expectation	Facility maternity registers
HL-1. 3c	Number of women referred for emergency obstetric care using the transport voucher scheme	women	0	180	34	—	18.9 %	162	90%	Below expectation	Voucher register
HL-1. 4a	Number of children under 1 year who received DTP3/Penta3	children	0	3300	540	—	16.4 %	2890	87.6 %	Below expectation	Facility and outreach immunization registers

HL-1.4b	Number of outreach immunization sessions conducted	sessi ons	0	180	34	—	18.9 %	174	96.7 %	Belo w exp ectati on	Outreach session records
HL-1.5a	Percentage of supported facilities submitting KHIS monthly reports on time	%	64	90	92	—	102.2 %	92	102.2 %	On track	KHIS reporting- rate dashboard
HL-2.1a	Number of community health volunteers (CHVs) trained and kitted	CHV s	0	220	10	—	4.5%	220	100%	On track	CHV training records
HL-2.2a	Number of people reached with community health education	peopl e	0	1800 0	3200	—	17.8 %	1750 0	97.2 %	Belo w exp ectati on	CHV and community dialogue registers
HL-2.3a	Number of children 6-59 months screened for acute malnutrition using MUAC	childr en	0	9000	1650	—	18.3 %	8800	97.8 %	Belo w exp ectati on	CHV MUAC registers
HL-2.3b	Number of children with acute malnutrition referred to treatment	childr en	0	650	120	—	18.5 %	625	96.2 %	Belo w exp ectati on	CHV referral slips

The Activity closed with all four outcome coverage indicators above target: ANC4+ reached 46% against a 45% target (baseline 28%), skilled delivery 61% against 60% (baseline 41%), Penta3 coverage 81% against 80% (baseline 62%), and caregivers naming three child danger signs 61% against 60% (baseline 31%); timely KHIS (Kenya Health Information System) reporting reached 92% against a 90% target (baseline 64%). Output targets were met or nearly met at life-of-activity level: 122 health workers trained in EmONC (emergency obstetric and neonatal care), IMCI (Integrated Management of Childhood Illness) or essential newborn care (101.7% of the 120 target, 71 female and 51 male), all 14 facilities equipped with MNCH (maternal, newborn and child health) kits (100% of target), and 220 CHVs trained and kitted (100% of target, 141 female and 79 male).

Results below expectation: 78 mentorship visits against a target of 84 (92.9% of target); 3,940 first ANC visits against 4,200 (93.8%); 1,800 skilled deliveries against 2,100 (85.7%); 162 emergency obstetric care voucher referrals against 180 (90%); 2,890 children receiving DTP3/Penta3 against 3,300 (87.6%, 1,445 female and 1,445 male); 174 outreach sessions against 180 (96.7%); 17,500 people reached with health education against 18,000 (97.2%, 10,850 female and 6,650 male); 8,800 children screened with MUAC against 9,000 (97.8%, 4,489 female and 4,311 male); and 625 malnourished children referred to treatment against 650 (96.2%). The officer's narrative attributes the missed count targets to earlier floods, the nurses' strike and cold-chain failures; coverage rates still reached or exceeded their targets. No indicator was flagged as not calculable.

Highlights (life-of-activity):

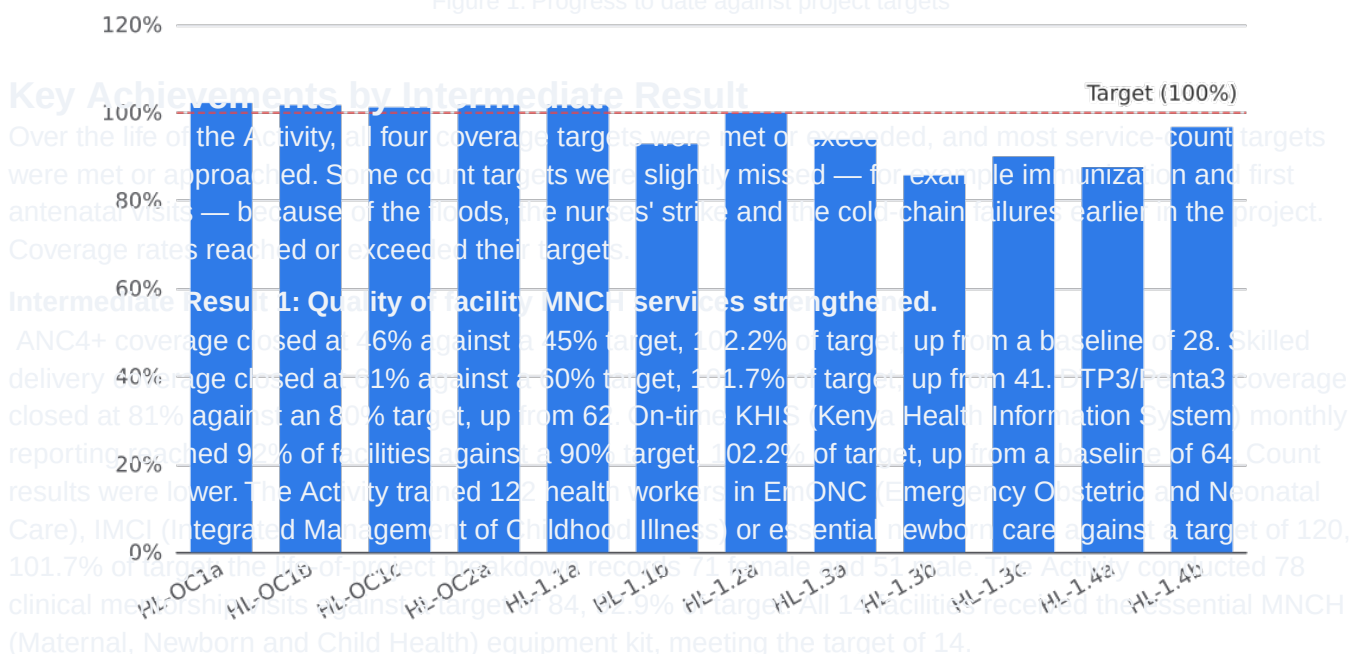
Sex disaggregation of life-of-activity people-level results: health workers trained 71 female, 51 male; CHVs trained 141 female, 79 male; children immunized with DTP3/Penta3 1,445 female, 1,445 male; people reached with health education 10,850 female, 6,650 male; children screened with MUAC 4,489 female, 4,311 male. First ANC visits are recorded as 3,940 female and 0 male. Skilled deliveries, voucher referrals, outreach sessions and malnutrition referrals carry no sex breakdown in the verified findings.

Data quality notes:

No indicator findings carried quality flags. The HL-OC2a caregiver-knowledge result comes from a KAP spot

survey with n=420 and covers two reporting periods; the officer noted low turnout in two villages during a week of hot weather, and those villages were revisited. Timey-reporting figures are drawn from the KHIS reporting-rate dashboard.

Figure 1. Progress to date against project targets



Intermediate Result 2: Service utilisation increased

The Activity recorded 3940 first ANC visits against a target of 4200, 93.8% of target. It recorded 1800 skilled deliveries against a target of 2100, 85.7% of target. The transport voucher scheme referred 162 women for emergency obstetric care against a target of 180, 90% of target. The Activity immunized 2890 children under one year with DTP3/Penta3 against a target of 3300, 87.6% of target; the life-of-project breakdown records 1445 female and 1445 male. It conducted 174 outreach sessions against a target of 180, 96.7% of target. The outreach immunization results are grounded in the verified monthly outreach session logs (e.g. 06-A1.4-outreach-session-log).

Intermediate Result 3: Community health capacity expanded.

The Activity trained and kitted all 220 community health volunteers against a target of 220, 100% of target; the life-of-project breakdown records 141 female and 79 male. The final KAP survey found 61% of caregivers could name at least three child danger signs against a 60% target, 101.7% of target, up from a baseline of 31. The endline KAP spot survey had a sample of n=420. The Activity reached 17500 people with community health education against a target of 18000, 97.2% of target; the life-of-project breakdown records 10850 female and 6650 male. CHVs screened 8800 children aged 6–59 months for acute malnutrition with MUAC against a target of 9000, 97.8% of target; the life-of-project breakdown records 4489 female and 4311 male. They referred 625 children with acute malnutrition to treatment against a target of 650, 96.2% of target.

One data-quality caveat applies. The final KAP survey fell in a week of hot weather with low turnout in two villages, and the two villages were revisited. Count results for the final month alone were smaller because the month closed training, equipment and CHV activity.

Intermediate Result 1: Quality Facility-Based MNCH Services

Facility-based maternal, newborn and child health (MNCH) services strengthened across all 14 supported facilities, and all service-coverage outcomes closed above target. Over the life of the Activity, the project trained 122 health workers in EmONC (emergency obstetric and neonatal care), IMCI (Integrated

The Activity completed its environmental mitigation obligations under the initial environmental examination (IEE) and recorded no environmental incidents during the life of the project. A final waste management audit covered all 14 supported facilities. The audit found all sharps containers in place and the placenta pits in good condition. The project's waste management checklist was handed to the sub-county public health officers, supporting continued county oversight of clinical waste after the award closed. Field report, August 2026 (LOCHA programme team). No remediation steps were required, as the final audit identified no non-compliance. No separate environmental monitoring evidence files were supplied for this section; the compliance status above rests on the recorded audit findings and the officer's closing report.

Collaboration, Learning and Adaptation

The Activity worked throughout with county and sub-county health structures, and this collaboration shaped how delivery adapted. Facility in-charges and records officers met monthly with the sub-county health records officers for data review and KHIS (Kenya Health Information System) audits in all 14 facilities; timely reporting rose from 66 percent of facilities in March to 92 percent in August against a 90 percent target (102.2% of target at endline). Sub-county teams joined delivery directly: the sub-county commander arranged a police escort for outreach after insecurity near Loruugum, the county pharmacist co-signed pre-agreed equipment kit lists, and the sub-county health management team signed the final asset handover certificates.

Monthly mentorship after classroom training changed practice. The project recorded that on-site mentorship after training improves use of partographs and neonatal resuscitation, so trainers paired every training cohort with on-site mentorship visits to the 14 facilities. Peer supervision by experienced CHVs (community health volunteers) kept newly trained volunteers active, and linking CHV pregnancy registers to facility ANC registers helped women return for the fourth visit.

The project adapted its community delivery methods to local conditions. Dialogues timed to market days and water-point visits reached more men. During drought, screening at water points found more children than house-to-house visits, and CHV screening days moved to water points and market days. Fixed outreach calendars shared with CHVs increased turnout at outreach sessions. When flooding cut the Kalokol and Lowarengak roads, routes were re-planned and children missed during a national Penta vaccine stock-out were recalled in following weeks. When two solar refrigerators failed, vaccines moved temporarily to the nearest working cold chain and the units were repaired within eight days.

The project also adapted its handover approach. Handing tools and registers to the county in the final month is smoother when the sub-county team has joined the monthly data review since the start; the August handover nevertheless needed more signatures than expected, and the sub-county health management team signed the asset handover certificates. The final KAP survey fell in a week of hot weather with low turnout in two villages; the project revisited the two villages to complete the survey.

What would you do differently? The records point to two adjustments: start-up procurement and nurse-mentor recruitment took longer than planned and partly covered facility services in the first weeks, so earlier recruitment and pre-agreed procurement would have allowed full service support from month one; and handover preparation, including signatures for solar fridges and registers, would be built into monthly joint reviews from the start rather than concentrated in the final month.

Sustainability and Transition

The Activity closed with all equipment, community health systems and supervision responsibilities handed to the County Government of Turkana, so the gains described in 'Key Achievements by Intermediate Result' rest on county-owned assets and structures.

Assets transferred.

The project delivered and installed the full essential MNCH equipment kit — delivery sets, neonatal

